INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Cisplatin every 3 weeks + head and neck radiotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00387
Version 5

> Cisplatin every 3 weeks × 3 with radiation.

Treatment stages

01 **Cisplatin with radiotherapy**
3 cycles · 3 weeks apart

02 **Recovery and follow-up**
After concurrent treatment

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology promptly for fever, sudden illness, persistent vomiting, much less urine, severe mouth pain or inability to drink. Report hearing change, ringing in the ears or new numbness before the next dose. Severe breathing difficulty, chest pain or collapse is an emergency.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

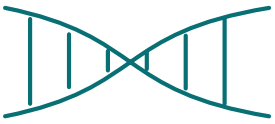
How each part of treatment works

Cisplatin and radiotherapy work together to treat locally advanced head and neck cancer. Your team explains the treatment aim, likely benefit and alternatives for your diagnosis.

PLATINUM CHEMOTHERAPY

Cisplatin

Cisplatin



Damages cancer-cell deoxyribonucleic acid (the cell's genetic material) (DNA)

Cisplatin forms links in DNA that make it harder for cells to repair and divide. During chemoradiation it also increases the effect of radiotherapy. Healthy cells can be affected, which is why kidney, blood, hearing and nerve checks matter.

DNA damage → Cell repair disrupted → Cancer growth reduced

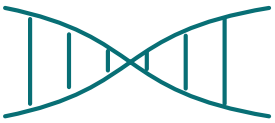
What to watch for

Report reduced urine, persistent vomiting, tinnitus, hearing change or new tingling.

LOCAL TREATMENT

Radiotherapy

Radiotherapy



Damages cancer cells in the planned treatment area

Radiotherapy directs radiation at a carefully planned area. It is delivered on its own appointment calendar. Effects depend on which tissues receive radiation and can build up during the course or continue after it finishes.

Treatment area planned → Repeated local treatment → Recovery and late-effect review

What to watch for

Report difficulty swallowing, weight loss, mouth pain, skin breakdown or problems managing saliva.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding your diagnosis

Keep your pathology and agreed treatment goal with your records.

Site, type and stage

Ask which part of the head or neck the cancer started in, what the biopsy shows and how far it has spread. Site, stage, previous treatment and your health affect the options. HPV/p16 testing is relevant to some sites; it is not the same test as PD-L1.

Which pathway applies?

Locally advanced stage III–intravenous (IV) squamous head and neck cancer. This guide should match the named treatment on your prescription.

What tumour tests mean

Your team explains any HPV/p16, Epstein–Barr virus or other tumour tests relevant to your cancer site. Not every head and neck cancer needs the same tests; the medicine safety checks are a separate matter.

Understanding the evidence

Cisplatin is used with radiotherapy under the exact NCCP indication and schedule.

RTOG 91-11 supports concurrent cisplatin-radiotherapy for preservation and local control of the larynx in its selected laryngeal-cancer population. It did not show an overall-survival advantage across its three groups. The long-term report also raised concerns about deaths unrelated to larynx cancer. These findings cannot be presented as a survival benefit across every head and neck site.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment stages

Treatment stages

01 **Cisplatin with radiotherapy**
3 cycles · 3 weeks apart

02 **Recovery and follow-up**
After concurrent treatment

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

3 CYCLES · 3 WEEKS APART

Cisplatin with radiotherapy

1 2 3

Cisplatin 100 mg/m² on Day 1 every 21 days for three planned cycles during radiotherapy. Cisplatin is given before radiation on the same day. If radiotherapy is cancelled, cisplatin is held until radiation resumes; the team updates both calendars.

AFTER CONCURRENT TREATMENT

Recovery and follow-up

Recovery | Individual review

Mouth, skin, swallowing, energy and hydration may need support after radiation ends. The team arranges response assessment and follows hearing, kidney or nerve problems. Any further chemotherapy is a separate prescription.

Dates and treatment decisions come from your team. A delayed or omitted dose does not count as treatment received.

BASELINE → TREATMENT → FOLLOW-UP

My treatment, tests and checks

Source-required blood tests are distinguished from tests ordered for a particular symptom or disease plan. Imaging is not interchangeable with blood safety tests.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00387 v5: official regimen

Tests in your plan

Full blood count, kidneys and liver

WHEN
Before every cisplatin cycle

Support safe treatment and identify problems needing review.

Hearing assessment

If part of your plan

WHEN

Audiology when indicated; report hearing or tinnitus changes before dosing.

Checks a medicine-specific risk that symptoms alone may miss.

THE WHOLE TREATMENT JOURNEY

Cisplatin every 3 weeks + head and neck radiotherapy: treatment and checks

Cisplatin 100 mg/m² on Day 1 every 21 days for three planned cycles during radiotherapy. Cisplatin is given before radiation on the same day. If radiotherapy is cancelled, cisplatin is held until radiation resumes; the team updates both calendars.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visit	Between visits	Phase / dose review	After treatment
Blood and organ tests NCCP	Blood count, kidney and liver function; audiology and measured creatinine clearance when indicated. Review hearing, tinnitus, nerve symptoms and medicines. Dental and nutrition preparation and a swallowing plan are important before head and neck radiotherapy.	Blood count, kidney and liver function before every cisplatin cycle. Report hearing or tinnitus and nerve changes before each dose; audiology is repeated when indicated. Review weight, drinking, swallowing, mouth and skin symptoms during the radiation course.	Contact the team if symptoms develop; extra tests may be needed before the next visit.	Recheck relevant results after a hold or changed plan; previous toxicity remains relevant.	Follow kidney, mineral, hearing and nerve problems and continuing radiation effects. Dental, nutrition and swallowing care may remain necessary after the last dose. Scans, examinations and any thyroid or other late-effect tests are individualised by the radiation and oncology teams; no universal scan or blood tumour-marker schedule is specified here.
Radiotherapy NCCP	Planning appointment and separate radiation calendar.	Cisplatin before same-day radiation; hold cisplatin if radiation is cancelled.	Attend radiation on its own calendar; no assumption of weekend sessions.	Coordinate interruptions and recovery between both teams.	Late effects and response follow-up belong to the radiation plan.

Care / check	Before starting	Treatment visit	Between visits	Phase / dose review	After treatment
Treatment and appointments NCCP	Confirm the exact diagnosis, intent, prescription and calendar.	Cisplatin is given by IV infusion with fluids before and after, prescribed mineral replacement and anti-sickness medicines. The unit confirms infusion and hydration times and checks that radiotherapy is proceeding that day. Allow time for both departments.	Use the prescribed support plan and contact route.	Mouth, skin, swallowing, energy and hydration may need support after radiation ends. The team arranges response assessment and follows hearing, kidney or nerve problems. Any further chemotherapy is a separate prescription.	Record the final dose and follow-up appointments.
Eating, mouth and swallowing NCCP	Agree oral care and assess weight, intake and swallowing; arrange dental review before head/ neck radiation.	Review pain, weight, hydration and safe swallowing.	Report poor intake, choking, weight loss or inability to drink promptly.	Dietitian and speech/ language plans adapt to symptoms and recovery.	Continue dental, nutritional and swallowing follow-up as needed.
Imaging and tumour assessment NCCP	Confirm staging and any relevant tumour tests; these are distinct from blood safety tests.	Discuss symptoms and treatment response.	The team arranges scans or examinations when indicated.	Use the agreed assessment to guide the next phase.	Record scan and clinic dates; this regimen does not impose a single timetable for everyone.

What each check is for

1 Cisplatin with radiotherapy

3 cycles · 3 weeks apart

Cisplatin and radiotherapy work together to treat locally advanced head and neck cancer. Your team explains the treatment aim, likely benefit and alternatives for your diagnosis.

Ask / act: Confirm the current phase, prescription, tests and support plan.

2 **Recovery and follow-up**

After concurrent treatment

Cisplatin and radiotherapy work together to treat locally advanced head and neck cancer. Your team explains the treatment aim, likely benefit and alternatives for your diagnosis.

Ask / **act:** Confirm the current phase, prescription, tests and support plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before the first visit

Confirm the exact regimen, treatment aim, baseline tests, transport, support medicines and actual urgent contact numbers. Bring any hearing concerns, swallowing problems and medicine list.

STEP 2

Time in the unit

Cisplatin is given by IV infusion with fluids before and after, prescribed mineral replacement and anti-sickness medicines. The unit confirms infusion and hydration times and checks that radiotherapy is proceeding that day. Allow time for both departments.

STEP 3

Two departments, one calendar

Radiation appointments are separate from medicine visits. Keep one calendar showing both departments and tests; tell the team promptly about a cancelled session.

STEP 4

Practical help

Tell the team about travel, cost, work, caring responsibilities, communication or reading needs. Ask whether a support person or interpreter can attend and how treatment changes will be explained.

STEP 5

Leaving the unit

Check the next visit and blood-test dates, support medicines and who to call if unwell. Have the team replace example phone numbers before using the wallet card.

Recovery, assessment and longer-term care

Treatment completion and recovery are different milestones.

Continuing symptoms

Follow kidney, mineral, hearing and nerve problems and continuing radiation effects. Dental, nutrition and swallowing care may remain necessary after the last dose. Scans, examinations and any thyroid or other late-effect tests are individualised by the radiation and oncology teams; no universal scan or blood tumour-marker schedule is specified here.



Cancer assessment

Your team sets examinations and imaging around the disease, treatment and symptoms. Record who arranges each test and who explains the result. An unbooked scan is not automatically due at a fixed week because a general guide says so.

Information for other clinicians

Keep the regimen name, last treatment date, ongoing medicines and relevant alert card. Tell clinicians about earlier immune treatment or head and neck radiation even after the course has ended.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Cisplatin	 **Day 1** Cisplatin precedes the same-day radiation session.
Radiotherapy course	 **Days 1–21** Separate radiation calendar; this band shows overlap, not treatment every day.
Mouth, fluids and nutrition	 **Days 1–21** Symptoms can build during treatment and continue afterwards.

Treatment sequence: Cisplatin 100 mg/m² on Day 1 every 21 days for three planned cycles during radiotherapy. Cisplatin is given before radiation on the same day. If radiotherapy is cancelled, cisplatin is held until radiation resumes; the team updates both calendars.

Baseline and ongoing checks: Blood count, kidney and liver function; audiology and measured creatinine clearance when indicated. Review hearing, tinnitus, nerve symptoms and medicines. Dental and nutrition preparation and a swallowing plan are important before head and neck radiotherapy. Blood count, kidney and liver function before every cisplatin cycle. Report hearing or tinnitus and nerve changes before each dose; audiology is repeated when indicated. Review weight, drinking, swallowing, mouth and skin symptoms during the radiation course.

Appointments and administration: Cisplatin is given by IV infusion with fluids before and after, prescribed mineral replacement and anti-sickness medicines. The unit confirms infusion and hydration times and checks that radiotherapy is proceeding that day. Allow time for both departments.

Recovery and follow-up: Follow kidney, mineral, hearing and nerve problems and continuing radiation effects. Dental, nutrition and swallowing care may remain necessary after the last dose. Scans, examinations and any thyroid or other late-effect tests are individualised by the radiation and oncology teams; no universal scan or blood tumour-marker schedule is specified here.

> **Why these bands stay broad:** The days show a usual treatment pattern, not a prediction of when symptoms will happen. Your team confirms dates and any changes. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00387 v5: official regimen ↗

> **There are no guaranteed "safe days".** Your prescription and team's instructions take priority. Contact the team about new symptoms rather than waiting for a scheduled visit.

Between-treatment notes

Day 1: Cisplatin then radiation

Safety review, anti-sickness medicines, fluids and cisplatin; radiation follows as arranged. Confirm both appointments; tell staff if radiation has been cancelled.

Day 2–21: Radiation and support continue

Follow the separate radiation calendar, oral care, drinking and nutrition plan. Call for symptoms early and arrange blood tests before the next cisplatin.

Support around the medicines

1  **One written medicine plan**

Ask for one schedule showing prevention medicines, rescue treatment and what to do if symptoms persist. Do not add, double or restart medicines from an old prescription.

2 **Dental and nutrition referral before starting**

NCCP 00387 specifically recommends dentistry and nutrition consultation before treatment. It also encourages oral hydration for 24 hours after cisplatin, adapted to your individual fluid plan.

3 **Hydration and kidney protection**

Cisplatin needs prescribed fluids before and after infusion. Follow the team’s drinking advice and report much less urine, vomiting or inability to drink. Do not change fluid intake or diuretics without advice.

4  **Food, fluids and weight**

A dietitian can tailor food texture, supplements and calorie or protein needs. Tell the team early if eating or drinking becomes difficult; do not wait for substantial weight loss. Fluid targets depend on kidney and heart health.

5 **Oral and dental care**

Use the mouth-care plan provided by the team and tell them about ulcers, thrush, bleeding or pain. Inform your dentist about cancer treatment. Before any dental procedure, confirm timing with the oncology team.

6 **Swallowing and communication**

A speech and language therapist can assess swallowing and advise exercises or safer food and drink textures. Follow the individual assessment; a general guide cannot decide which textures are safe for you.

7

Fertility, pregnancy and medicines

Discuss fertility and pregnancy precautions before treatment and the medicine-specific duration of contraception afterwards. Ask about breastfeeding. Bring a full medicine, supplement and allergy list; check before adding over-the-counter or herbal products.

Treatment choices and your priorities

The consent conversation explains the boundaries of the plan.

Benefit and alternatives

Cisplatin and radiotherapy work together to treat locally advanced head and neck cancer. Your team explains the treatment aim, likely benefit and alternatives for your diagnosis. Ask how alternatives compare for your health, goals and previous treatment. A study result cannot predict an individual outcome.

A pause is a treatment decision

The clinician may delay, change or stop treatment because of symptoms, test results or your priorities. Record which component is affected and what is needed before restarting. Do not restart from an old prescription.

Combining treatment approaches

Radiotherapy treats a planned local area while medicine supports the cancer treatment. The radiation prescription, number of sessions and late-effect plan are agreed separately.

Preparing for and receiving treatment

Plan the appointments and support around the prescribed schedule.

If an appointment changes

If radiation is cancelled, cisplatin is withheld until radiation resumes. Ask both departments to confirm revised dates; do not assume the chemotherapy visit proceeds unchanged.

What the record shows

Write the actual treatment date, phase and any change. The wallet card is a brief record; the passport holds the fuller history. It does not replace the hospital treatment record.

Between visits: symptoms and medicines

Use the agreed home plan and contact pathway.

Symptoms have no fixed calendar

Some effects appear soon after treatment and others build over weeks. Do not wait for the next appointment if a symptom is new, severe or worsening.

A medicine plan you can use

Keep prevention, rescue and regular medicines clearly separated on one written list. Record what you have taken and the response, so the team can adjust it. Do not double a missed dose unless your prescriber tells you to.

Mouth, teeth and jaw care

Dental preparation and gentle daily care reduce avoidable problems.

Before treatment

Arrange dental assessment before head and neck radiation. The dental and oncology teams decide what treatment is needed and how much healing time is required; do not schedule extractions independently.

During treatment

Follow the prescribed brushing, fluoride, rinse and pain-relief advice. Report ulcers, infection, new bleeding, difficulty wearing dentures or worsening jaw opening. Avoid starting a new mouthwash without checking if the mouth is sore.

After head and neck radiation

Dry mouth can increase dental problems and some radiation effects last. Keep regular dental care and tell any future dentist that you have had head and neck radiation before invasive treatment. Your radiation/dental team plans any procedure.

Eating, drinking and maintaining strength

The aim is adequate intake and comfort during treatment.

Recognising an intake problem

Record weight and report food or drink becoming harder to manage. Pain, taste change, dry mouth or nausea can each need a different response; a dietitian can adapt the plan to you.

If ordinary food is not enough

The team may suggest food fortification, supplements or tube feeding when intake or swallowing safety requires it. A tube is a support option, not a judgement about how well you have tried to eat. Training and follow-up are needed.

Fluid needs are individual

Ask for a realistic drinking plan that accounts for kidney/heart problems and any feeding tube. Persistent vomiting, dizziness, markedly reduced urine or inability to drink needs urgent contact.

Swallowing, voice and communication

A speech and language therapist can help protect function.

Swallowing assessment

Report coughing or choking with food or drink, a wet-sounding voice after swallowing, food sticking or repeated chest infections. The team may need to assess aspiration risk and safe textures.

Exercises and rehabilitation

Do swallowing or jaw exercises only as taught for your situation. Treatment and recovery can change what is comfortable and safe. Ask for reassessment when symptoms change.

Communication needs

Tell the team about changes in speech, voice or hearing. Ask for written instructions, communication aids, an interpreter or help involving a support person.

Daily life and emotional support

Practical adjustments can make the plan easier to follow.

Energy and activity

Balance rest with manageable activity according to your ability and team advice. Ask for help if weakness, falls, sleep difficulty or pain interferes with daily life.

Smoking and alcohol

Ask for support to stop smoking and discuss alcohol with the team. Mouth irritation, medicine interactions and head and neck recovery can change what is safe for you.

Emotional and practical concerns

Tell the team about anxiety, appearance or intimacy concerns, finances, travel and caring responsibilities. Ask which local supports are available; resources can be added to your record when confirmed.

Your records and additional help

Keep essential instructions available on paper.

The passport and wallet card

The three-page companion gives the essentials. Use the passport for the agreed plan, visits, test results, symptoms and questions. The wallet QR opens this exact treatment's further information; it contains no personal clinical record.

Contacts before patient use

The phone numbers printed as examples must be replaced and checked by the treating service. Add the unit's daytime, out-of-hours and backup route; keep this information where you and a support person can find it.

If you prefer paper

Use the full printable handbook in its chapter order and write questions in the passport. You do not need to scan the QR or follow an internet link to use the essential treatment and safety information.

Questions worth asking

Which tests are required?

Blood count, kidney and liver function; audiology and measured creatinine clearance when indicated. Review hearing, tinnitus, nerve symptoms and medicines. Dental and nutrition preparation and a swallowing plan are important before head and neck radiotherapy. Blood count, kidney and liver function before every cisplatin cycle. Report hearing or tinnitus and nerve changes before each dose; audiology is repeated when indicated. Review weight, drinking, swallowing, mouth and skin symptoms during the radiation course.

What if a treatment is held?

The team records the reason and recovery or tests needed before restarting. Do not count an omitted dose as completed or restart from an old plan.

Try explaining the plan back: Explain which treatment phase you are in, what happens next, which tests are due and which symptoms would make you call today. Ask the team to clarify anything you cannot explain.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cisplatin with radiotherapy

Active medicines: Cisplatin

Emetogenic risk: high

Medicine	Dose / route	When
High-emetic-risk anti-sickness plan	Clinician/pharmacy to complete	Before platinum and for delayed symptoms as prescribed Use the current local/NCCP policy; reconcile prevention and rescue medicines into one schedule.
Cisplatin hydration and electrolyte plan	Clinician/pharmacy to complete	Before and after cisplatin; home fluids individualised Check kidney function, fluid balance and relevant comorbidities; do not issue an automated fluid prescription.

Additional phase-specific support

- Ask for one schedule showing prevention medicines, rescue treatment and what to do if symptoms persist. Do not add, double or restart medicines from an old prescription.
- Cisplatin needs prescribed fluids before and after infusion. Follow the team's drinking advice and report much less urine, vomiting or inability to drink. Do not change fluid intake or diuretics without advice.
- A dietitian can tailor food texture, supplements and calorie or protein needs. Tell the team early if eating or drinking becomes difficult; do not wait for substantial weight loss. Fluid targets depend on kidney and heart health.
- Use the mouth-care plan provided by the team and tell them about ulcers, thrush, bleeding or pain. Inform your dentist about cancer treatment. Before any dental procedure, confirm timing with the oncology team.
- A speech and language therapist can assess swallowing and advise exercises or safer food and drink textures. Follow the individual assessment; a general guide cannot decide which textures are safe for you.
- Discuss fertility and pregnancy precautions before treatment and the medicine-specific duration of contraception afterwards. Ask about breastfeeding. Bring a full medicine, supplement and allergy list; check before adding over-the-counter or herbal products.
- NCCP explicitly recommends dentistry and nutrition consultation before treatment and oral hydration for 24 hours after cisplatin, individualised to fluid status.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hearing / tinnitus

Cisplatin

Report ringing in the ears or hearing change before the next dose; effects can be lasting.

3 Blood cells / infection

Cisplatin

Fever, shivering, sudden illness, unusual bruising or bleeding needs prompt oncology advice.

5 Nausea / appetite

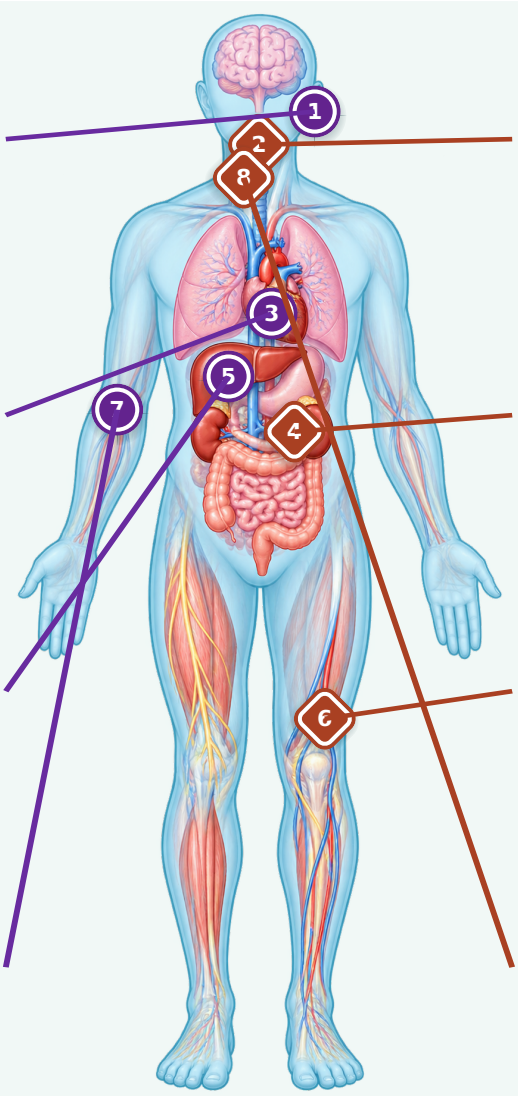
Cisplatin

Take the prescribed prevention and rescue medicines. Call if vomiting persists or fluids will not stay down.

7 Skin / treated area

Radiotherapy

Follow the radiation team’s skin plan. Report painful redness, peeling or a wound that is not healing.



2 Mouth / throat / saliva

Head and neck radiotherapy

Mouth pain, thick saliva, dry mouth or swallowing difficulty can limit drinking; seek help early.

4 Kidneys / fluids

Cisplatin

Much less urine, persistent vomiting or inability to drink needs urgent advice. Drip fluids and blood tests help protect kidneys.

6 Nerves / hands / feet

Cisplatin

Report numbness, tingling, balance problems or difficulty with fine tasks; symptoms can persist.

8 Teeth / jaw / swallowing

Head and neck radiotherapy

Dental review, nutrition and swallowing support continue through recovery. Report jaw stiffness, weight loss or choking.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact oncology promptly for fever, sudden illness, persistent vomiting, much less urine, severe mouth pain or inability to drink. Report hearing change, ringing in the ears or new numbness before the next dose. Severe breathing difficulty, chest pain or collapse is an emergency.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital’s temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00387 v5 · pSee regimen treatment, tests and supportive-care sections**
   https://healthservice.hse.ie/documents/6912/387_V5_CISplatin_100mgm2_with_radiotherapy (RT).pdf
2. **NCCP head and neck regimen catalogue**
   https://healthservice.hse.ie/staff/information-healthcare-workers/nccp/head-and-neck-sact-regimens/
3. **NCI: head and neck cancers**
   https://www.cancer.gov/types/head-and-neck/head-neck-fact-sheet
4. **NCI: oral complications of cancer therapies, patient version**
   https://www.cancer.gov/about-cancer/treatment/side-effects/mouth-throat/oral-complications-pdq
5. **NCI: nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
6. **HSE: cancer treatment side effects**
   https://www.hse.ie/conditions/cancer/if-you-have-side-effects/
7. **Irish Cancer Society: head and neck radiotherapy effects**
   https://www.cancer.ie/cancer-treatments/radiotherapy/side-effects-from-radiotherapy-to-the-head-and-neck-area
8. **RTOG 91-11: original larynx-preservation report, 2003**
   https://pubmed.ncbi.nlm.nih.gov/14645636/
9. **RTOG 91-11: long-term larynx-preservation report, 2013**
   https://pubmed.ncbi.nlm.nih.gov/23182993/
10. **NCCP antiemetics v8, Table 1 pp4–5**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf